

34. Infection-Related Glomerulonephritis · assets 1-9 / 12

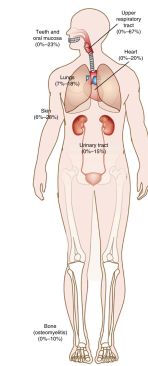


Fig. 34.2 Sites of infection in adult bacterial infection-related glomerulonephritis (IRGN). (With permission from Nasr SH, Padakrishnan J, D'Agati VD. Bacterial infection-related glomerulonephritis

Fig 34 2

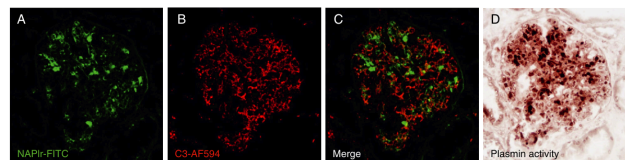


Fig. 34.5 Histology of glomeruli from a patient with poststreptococcal glomerulonephritis. The photomicrographs show glomerular staining for complement (C3), nephritic-associated plasmin receptor (NAPR), and plasmin deposits in the glomeruli of poststreptococcal glomerulonephritis. Though the glomeruli show staining for NAPR (fluorescein isothiocyanate, green) (A) and C3 (Alexa Fluor 594, red) (B), they are not colocalized (C). Plasmin activity assessed by *in situ* zymography was found to be positive and had a similar distribution to the NAPR staining in the glomeruli (D). (Images provided by Dr. Takashi Oda, Professor, Department of Nephrology and Blood Purification, Tokyo Medical University, Tokyo, Japan.)

Fig 34 5



Figure 1 is a line graph showing the course of four variables over 12 weeks. The x-axis is labeled 'Course in weeks' and ranges from -6 to 12. The y-axis represents levels. A blue shaded area from week -6 to -2 is labeled 'Streptococcal infection'. A yellow shaded area from week -2 to 0 is labeled 'Latent period'. The variables are: C3 level (blue line), Hemofiltration (grey line), MTN (green line), and ASD (red line). C3 level starts high, drops at week -2, and remains low. Hemofiltration starts low, peaks at week 1, and then declines. MTN starts low, peaks at week 1, and then declines. ASD starts low, peaks at week 4, and then declines.

Fig 34 6

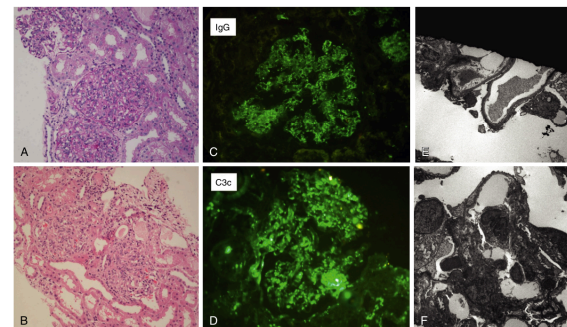
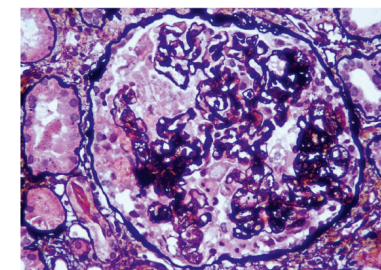


Fig. 34.8 Histology of glomeruli from a patient with poststreptococcal glomerulonephritis. (A and B) Periodic acid-Schiff and hematoxylin-eosin-stained sections, respectively, showing a proliferative, exudative pattern of injury. (C and D) From direct IF study showing peripheral and mesangial coarse granular deposits of IgG and C3c deposits being dominant. (E and F) From electron microscopy showing subepithelial humps. (Histopathology images have been provided by Dr. Swarnalatha Gowrishanker, Nephropathologist, Apollo Hospital, Hyderabad, India.)

Fig_34_8



Fig_34_9

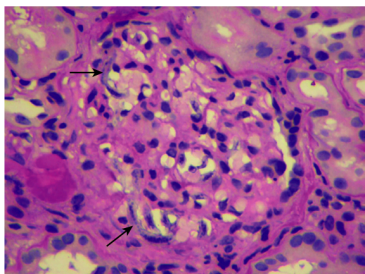


Fig. 34.10 Histology of a glomerulus from a patient with filarial glomerulonephritis. Light microscopy (PAS stain) of a kidney biopsy specimen with *black arrows* showing slender tube-like structures with parallel rows of nuclei representing microfilariae trapped within the capillaries of the glomerulus.²¹⁶ (Permission from Dr. Anila Abraham, Nephropathologist, Renopath, Chennai, India.)

Fig_34_10

[illegible]

Table_34_1

Table 34.2 Histologic Lesion or Glomerulonephritis Patterns in Infection-Related Glomerulonephritis

Histologic lesions/glomerulonephritis patterns

- Endocapillary glomerulonephritis
- Exudate glomerulonephritis
- Mesangial proliferative glomerulonephritis
- Membranoproliferative glomerulonephritis
- Necrotizing glomerulonephritis
- Crescentic glomerulonephritis
- Pauci-immune glomerulonephritis
- Collapsing glomerulonephritis
- Minimal change glomerulonephritis
- Sclerosing glomerulonephritis
- Membranous glomerulonephritis

Table_34_2